Amiodarone

Scope of Practice	EMT: Cardiac Arrest AP: Infusion for Tachycardia
Classification	• Antiarrhythmic
Usual Presentation	• 150mg/3ml ampule
Mechanism of Action	• Amiodarone is a wide-acting antiarrhythmic drug, mainly classified as class III. • It extends the action potential duration, lowers automaticity, and increases the refractory period in atrial, nodal, and ventricular tissues. • Its electrophysiological effects reduce irregular electrical activity (like ectopic beats), slow down electrical conduction, decrease the heart rate, and stabilize the SA and AV nodes. • Amiodarone also slightly reduces myocardial oxygen demand by decreasing inotropy (the strength of heart contractions).
Indications	• Cardiac Arrest (refractory VT or VT) • Rate control/ reversion of wide complex tachyarrhythmias
Contraindications	• Known allergy • VT of VF secondary to Tricyclic Antidepressant Overdose
Precautions	• None if the patient is in cardiac arrest • Hypotension • Sick sinus syndrome (without a pacemaker in place) • Previous 2nd or 3rd degree Heart Block (without a pacemaker in place)
Side Effects	• Hypotension • Lightheadedness • Arrhythmias – particularly bradyarrhythmia's
Adult Dose & Administration	**Cardiac Arrest:** • 1st Dose: 300mg IV/IO push • 2nd Dose: 150mg IV/IO push **Wide Complex Tachycardia:** • 150mg/10minutes • Dilute 150mg (3ml) of amiodarone with 100ml D5W and administer it over 10 minutes.
Pediatric Dose & Administration	• **Cardiac Arrest:** • 1st Dose: 5mg/kg IV/IO • Dose may be repeated twice (maximum 15mg/kg total)
Use in Pregnancy and Lactation	• Pregnancy: fetal harm has been demonstrated. Administration is not recommended unless significantly clinically indicated. • Amiodarone should not be withheld in cardiac arrest in pregnancy. • Lactation: May be administered during pregnancy, however the mother should be advised to stop breast feeding until advised by their physician.
Additional Information	• Administer into a large vein where possible. • If hypotension occurs slow or stop the rate of administration. • The full dose should be administered even if the rhythm reverts to sinus rhythm (unless hypotension of bradycardia develops)